

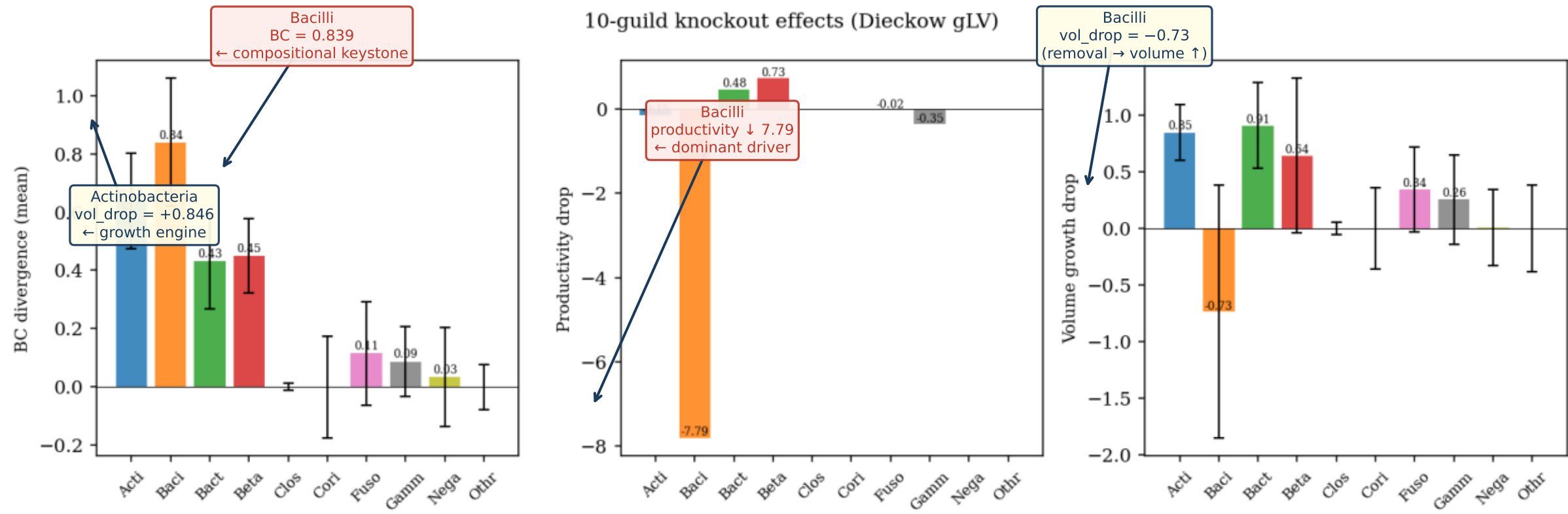
Guild-Level Dynamical Analysis

Keystone · Network Centrality · Relapse Dynamics · Prevention · Phase Diagram

Dieckow 2024 · 10-guild gLV · $N = 10$ patients · Nishioka (2026)

Guild-Level Keystone Analysis: Knockout BC Divergence

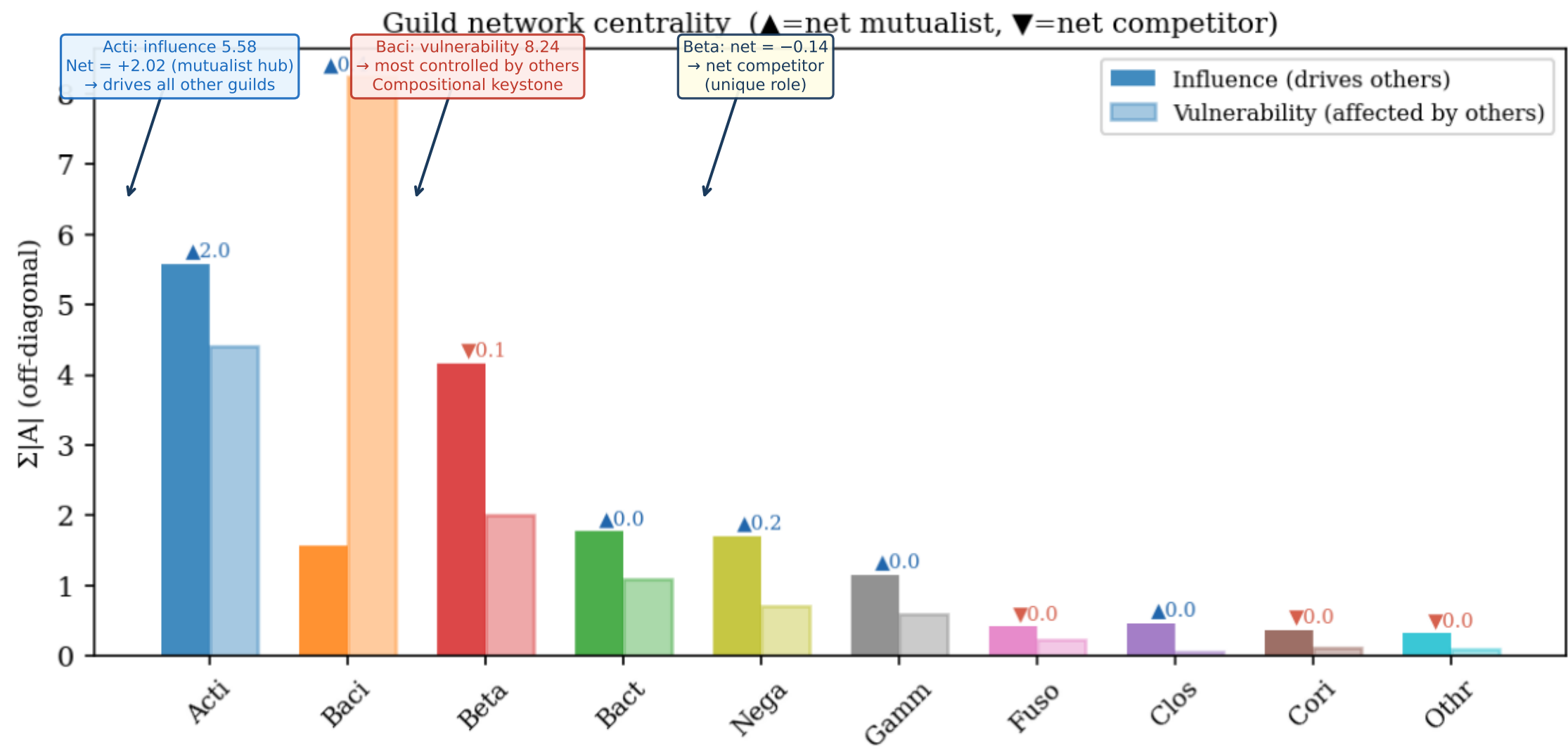
Remove one guild → simulate gLV 21 d → measure BC shift, productivity drop, volume change



Bacilli: highest BC divergence (0.839) — compositional keystone. Removal increases total volume → competitive suppressor. | Actinobacteria: vol_drop = +0.846 → growth engine. Strongest single interaction: Acti → Bacil A = +3.43.

Guild Network Centrality: Influence, Vulnerability, Net Role

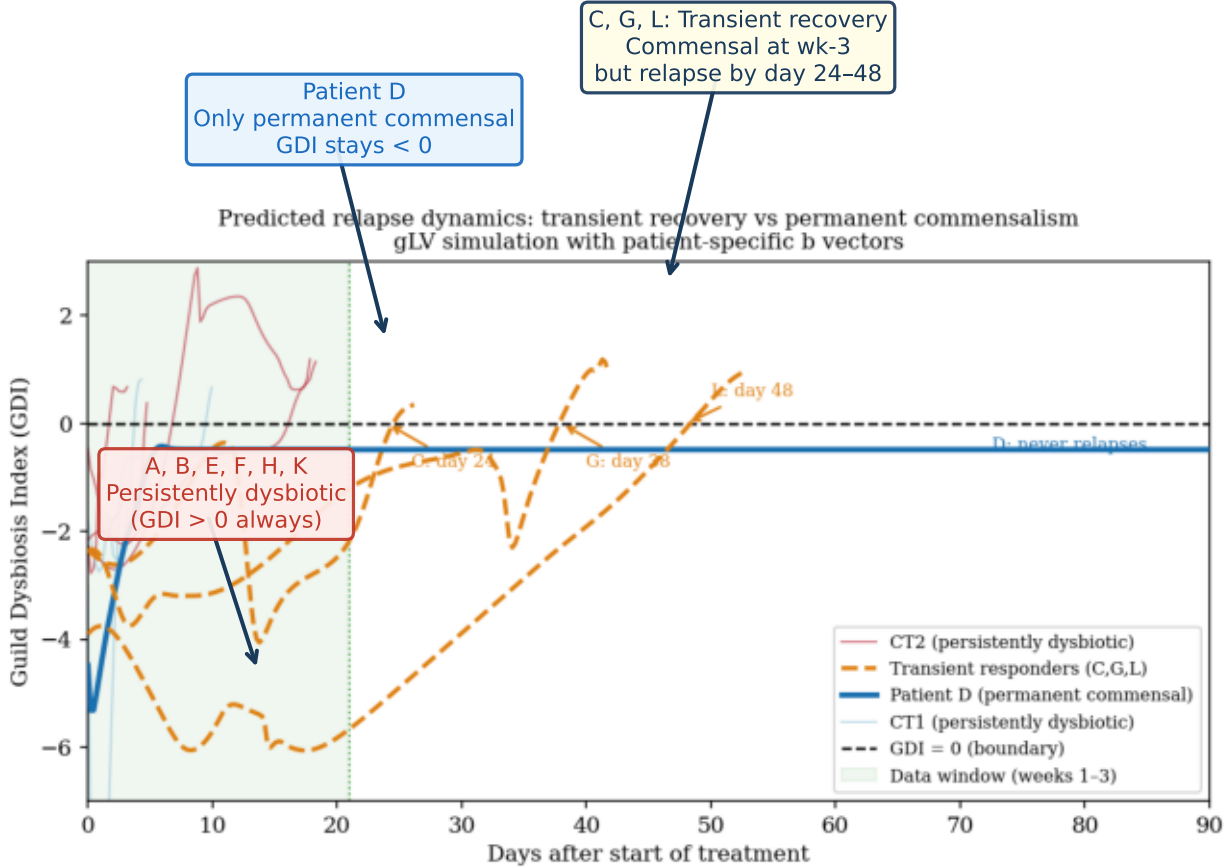
Influence = $\sum |A_{ji}|$ (how much guild i drives others) | Vulnerability = $\sum |A_{ij}|$ (how much others drive guild i)



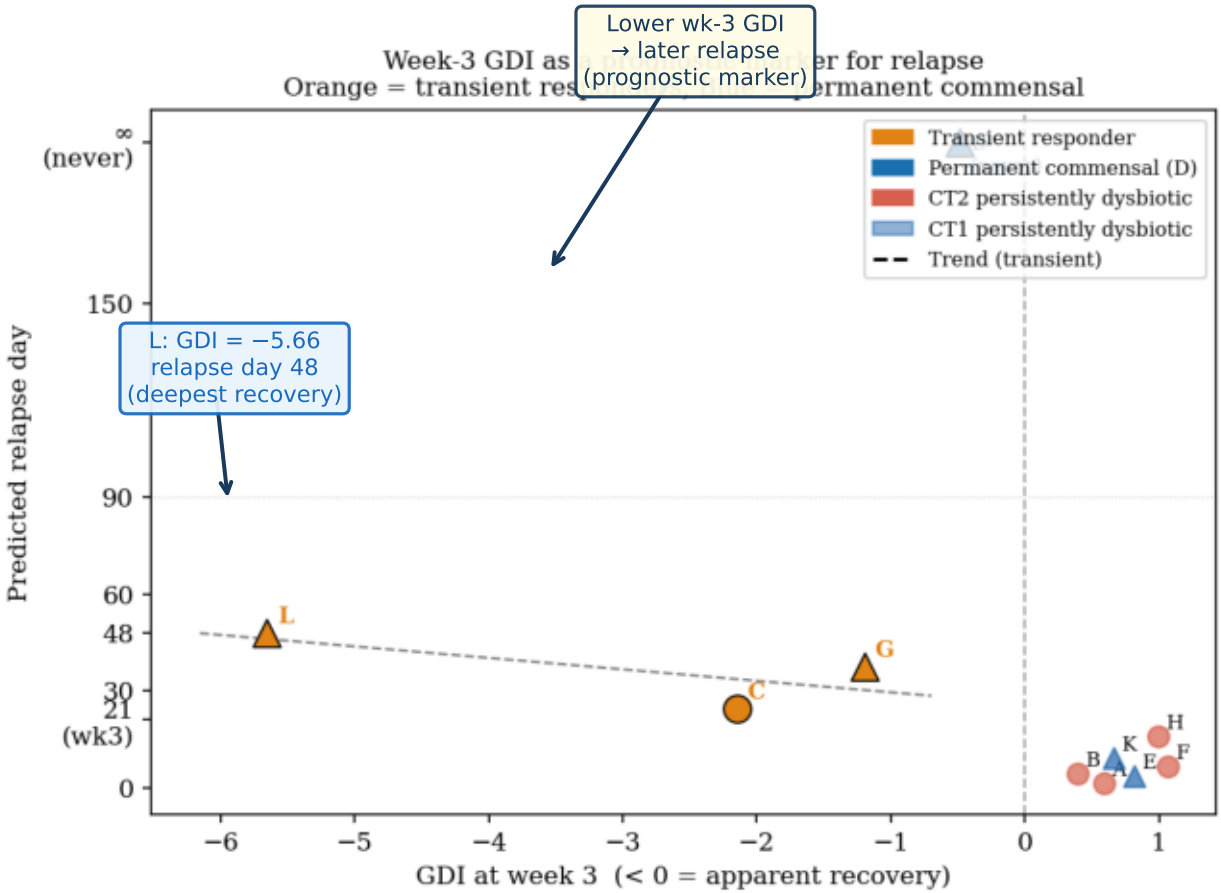
Actinobacteria: influence = 5.58 (highest), net = +2.02 → mutualist hub. Bacilli: vulnerability = 8.24 → most regulated by others. Betaproteobacteria: net = -0.14 → net competitor. A matrix encodes dysbiotic attractor via Acti-Baci-Beta triad.

Predicted Relapse Dynamics: Three Patient Archetypes

gLV simulation $t = 150$ d with patient-specific b vectors | $GDI = \log(\phi_{\text{dys}}) - \log(\phi_{\text{com}})$



(a) GDI trajectories to 90 d

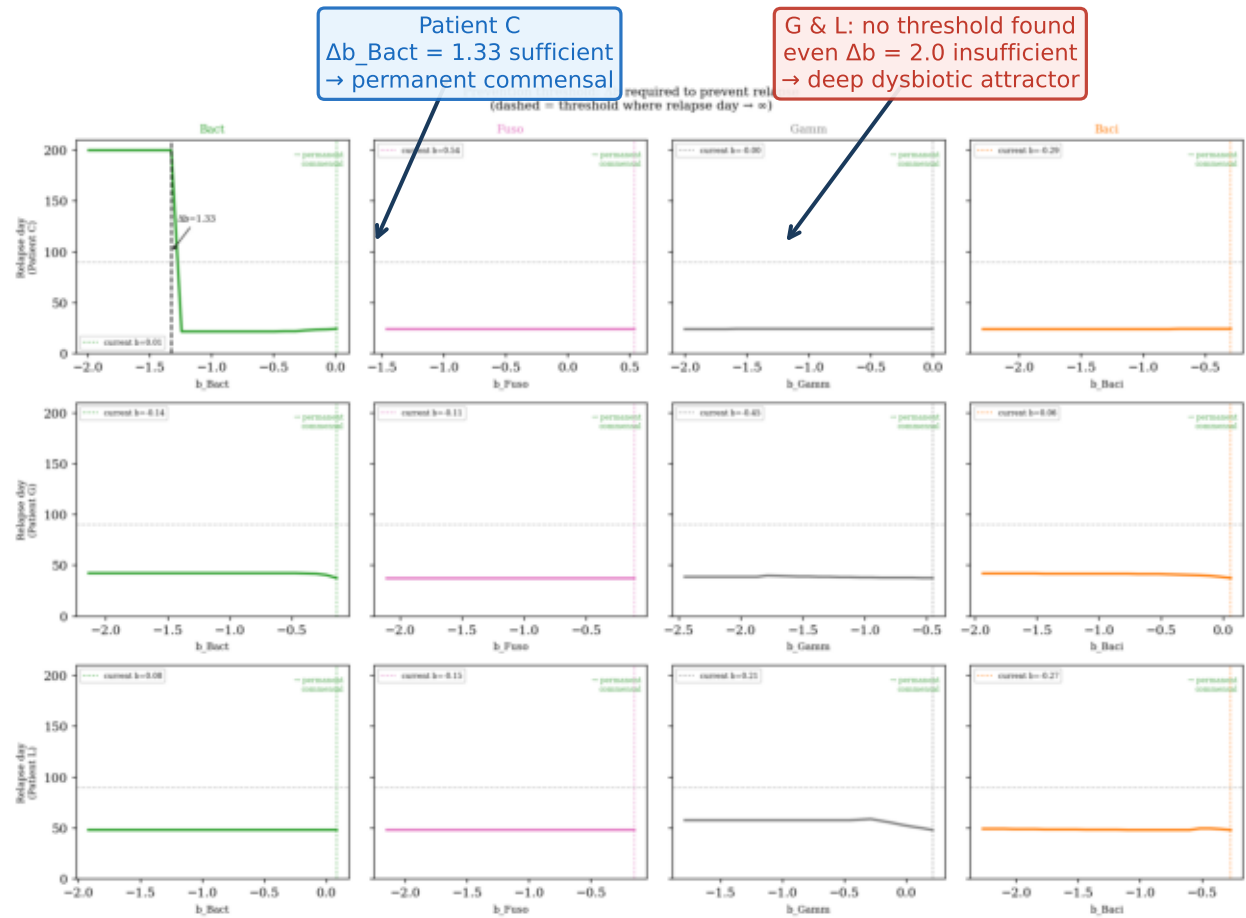


(b) Week-3 GDI as prognostic marker

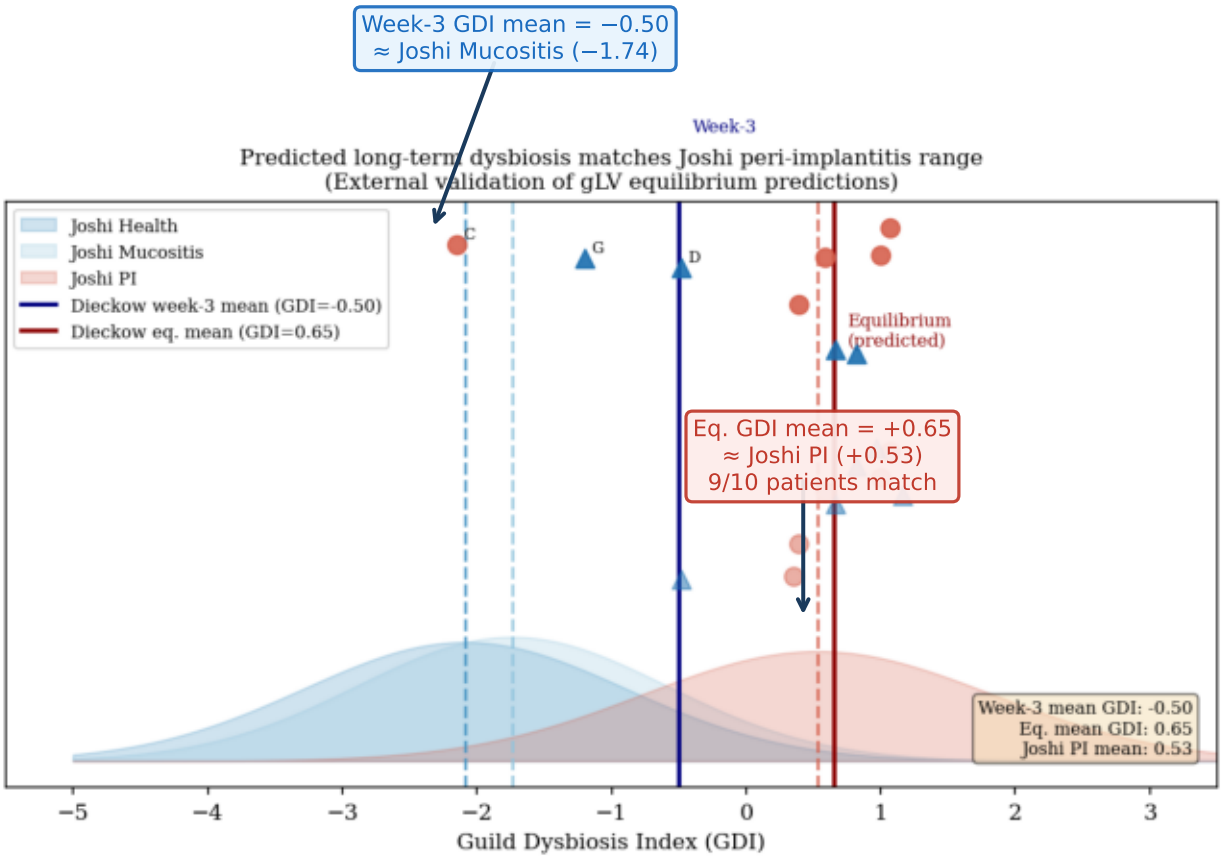
① Persistent dysbiotic (A,B,E,F,H,K): GDI > 0 throughout. ② Transient responders (C $\rightarrow$ 24d, G $\rightarrow$ 38d, L $\rightarrow$ 48d): commensal at wk-3, dysbiotic by day 50. ③ Permanent commensal (D only): stable GDI < 0 . $\rightarrow$ Week-3 GDI as prognostic marker: lower = longer remission.

Prevention Threshold & External Validation (Joshi 2025)

Min. Δb_i to prevent relapse (single-guild sweep) | Model eq. GDI vs peri-implantitis reference distribution



(a) Required Δb per guild for prevention

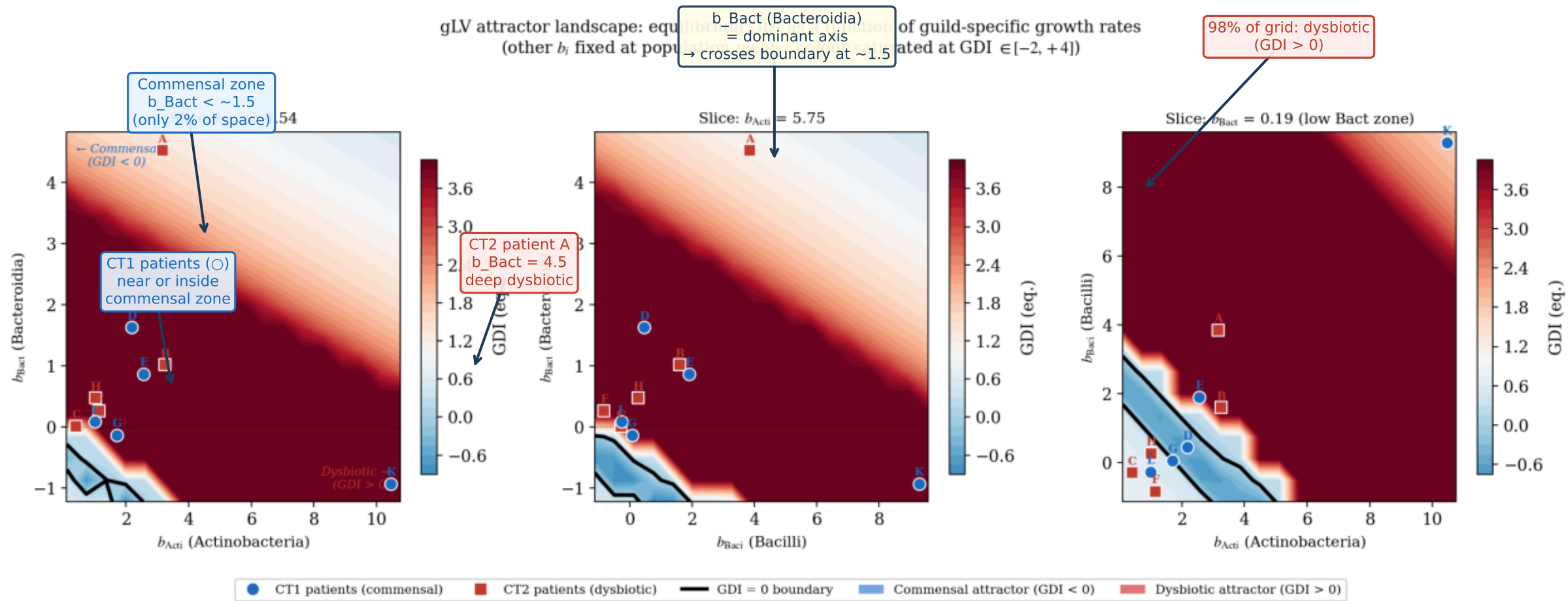


(b) External validation: Joshi 2025 peri-implant cohort

Prevention: only Patient C convertible by single-guild intervention ($\Delta b_{\text{Bact}} = 1.33$). G & L require multi-guild change → deep attractor. | Joshi validation: predicted eq. GDI = $+0.65 \approx$ Joshi PI mean $+0.53$ (9/10 patients; difference $< 0.12 <$ Joshi SD = 1.30).

3D Attractor Landscape: GDI = 0 Boundary in Growth-Rate Space

18³ grid, Acti × Baci × Bact, t = 80 d | Blue = commensal | Red = dysbiotic | Black = GDI = 0 boundary



98% of parameter space is dysbiotic ($GDI > 0$). Commensal region confined to $b_{Bact} < \sim 1.5$ — Bacteroidia growth rate is the dominant control variable. CT1 patients with low b_{Bact} cluster near boundary; CT2 patient A ($b_{Bact} = 4.5$) is deep in dysbiotic zone. → Structural attractor robustness — not initial-condition sensitive.